History Taking in Psychiatry

Psychiatry, Clinical Skills | Dr. Hussain M. Khrad (Consultant Psychiatrist, KFSHRC-Jeddah) | Source: `History-Taking.pdf` (43 slides)

> **Legend:** ⭐ = high-yield · *[added]* = not on slides. Images marked "Slide N" come from the lecture. The 2 MCQ answers and the case model answer come from the lecture's own answer slides.
>
> 🔗 **Bridge:**
>
> - This is the **first psychiatry skills lecture**. The **Mental State Examination** is the next step after the history (slide 10).
> - **Personality Disorders:** "premorbid personality" separates a lifelong PD from a **new illness** (a change). The "new onset after 40 = organic" rule recurs here.
> - **Medicine overlap (medical mimics):** thyroid disease, **Cushing's/Addison's**, **hyper/hypocalcaemia (PTH)**, low Na/K/Mg (all covered in your endocrine notes) can present as psychiatric illness.

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1. Title / Objectives

Domain	Objective	Likely tested as
Knowledge	Define a **mental disorder** and how diagnosis is made	Bereavement and social deviance are NOT disorders
Knowledge	⭐ List the **9 sections** of a complete history	Order of sections; premorbid personality
Knowledge	⭐ Recognise **organic red flags** and medical mimics	58-year-old + visual hallucinations + fluctuation = delirium
Skill	Set up an interview that builds rapport	L-shaped seating; open → closed questions
Skill	Screen the major disorders **bilingually** (English/Arabic)	GAD, panic, agoraphobia, social anxiety, phobia, OCD, psychosis
Skill	⭐ Structured **suicide and homicide** risk assessment	Previous attempt = strongest predictor
Skill	Manage the **difficult interview**	Guarded, hostile, mute, over-talkative, seductive
Values	Interview = **therapeutic act**; privacy and confidentiality; asking about suicide is a **duty, not a risk**	"Asking plants the idea" = myth

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2. Main Content (Lecture Order)

2.1 Introduction: Psychiatry and Mental Disorder (Slides 3–5)

Topic	Key points
Psychiatry	Medical specialty for the study, diagnosis, treatment and prevention of mental disorders: **affective, behavioural, cognitive, perceptual** abnormalities

Topic	Key points
Origin	Coined by **Johann Christian Reil, 1808**. Greek **psykhē (soul) + iatreia (healing)** = "medical treatment of the soul"
⭐ **Mental disorder**	A **syndrome** of clinically significant disturbance in **cognition, emotion regulation or behaviour**, reflecting a **dysfunction** in psychological, biological or developmental processes. Usually **significant DISTRESS or DISABILITY** in social/occupational/other functioning
⭐ **NOT a disorder**	(1) An **expectable or culturally approved response** to a stressor or loss, e.g. **bereavement**. (2) **Socially deviant behaviour** or individual–society conflict, **UNLESS** it results from a dysfunction in the individual
Diagnosis	**Syndrome-based**: signs and symptoms that **co-occur**, with a characteristic **course and treatment response**
Classification	**DSM-5-TR** (APA, 2022) · **ICD-11** (WHO), in effect since **2022**, superseding ICD-10

⭐ **No blood test or scan diagnoses most psychiatric disorders: THE HISTORY AND MENTAL STATE EXAM ARE THE INVESTIGATION.** The exclusions stop psychiatry being used to pathologise difference or dissent.

2.2 The Interview (Slides 7–10)

Goals (slide 7):

Clinical	Relational
Get the information needed for a **diagnosis**	Form a **therapeutic relationship** (rapport)
Understand the **PERSON** and their circumstances, not just the illness	Give information about the illness, plan and prognosis

The interview is **both a diagnostic tool and a therapeutic act**.

Setting up (slide 8):

Set the scene	Conduct
Professional dress; introduce yourself; greet the patient **by name**	Start with **OPEN-ENDED** questions, then focus
Reassure about **privacy and confidentiality**	Observe **non-verbal** behaviour; avoid excessive note-taking
Private, comfortable setting. ⭐ **L-SHAPED SEATING** at a suitable distance; **not face-to-face** (feels like an interrogation)	Decide who to start with (patient or relative) and **ask why the relative came**
Explain who you are, the purpose, the time needed	—

⭐ The 9 Sections of a Complete Psychiatric History (Slides 9–10)

Same order every time, so nothing is missed.

#	Section	Content
1	**Personal data**	Name, age, marital status, religion, residence, occupation, **handedness**
2	**Presenting complaint**	In the **patient's own words** + from a **reliable informant**
3	**History of present illness**	Onset, precipitant, symptoms **present AND absent**, help-seeking, response so far
4	**Past psychiatric history**	Admissions, episodes, **ECT**, diagnoses, treatment responses

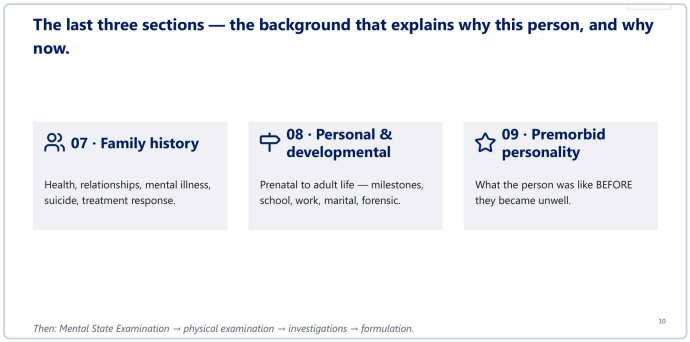
#	Section	Content
5	**Past medical & drug**	Medical, surgical, and **psychiatric side-effects of medications**
6	**Substance history**	For each substance: onset, pattern, route, effects, withdrawal
7	**Family history**	Health, relationships, mental illness, **suicide**, treatment response
8	**Personal & developmental**	Prenatal → adult life: milestones, school, work, marital, **forensic**
9	**Premorbid personality**	What the person was like **BEFORE** they became unwell

Then: Mental State Examination → physical examination → investigations → **formulation**.

Mnemonic *[added]*: "People Present Histories, Past Medical Substances, Family Personal Premorbid" (P-P-H-P-M-S-F-P-P).



Slide 9: History framework, sections 1–6



Slide 10: Sections 7–9

> 🔁 **Recap: The interview.** Rapport + diagnosis. Private room, L-shaped seating, open → focused questions, watch non-verbal cues. 9 sections in fixed order, then MSE, examination, investigations, formulation.
>
> ▶️ **Up next: Each section in detail, with screening scripts.**

2.3 Personal Data, Complaint and Informant (Slide 12)

- Record the complaint **in the patient's own words**, and also **from a reliable informant**.
- ⭐ **Reliable informant:**
  1. **close to, and living with**, the patient
  2. **no conflict of interest**
  3. **no psychiatric disorder** themselves
- ⭐ **Collateral history is essential**, especially where **insight is limited**: **mania, psychosis, dementia**.

2.4 History of Present Illness (Slide 13)

Build the story	Then ask about
Onset, any stressor/precipitant	**How the patient sought help**
Symptoms **PRESENT**, with full analysis	**Response to treatment** so far
⭐ Symptoms **ABSENT**, to exclude the differentials	—

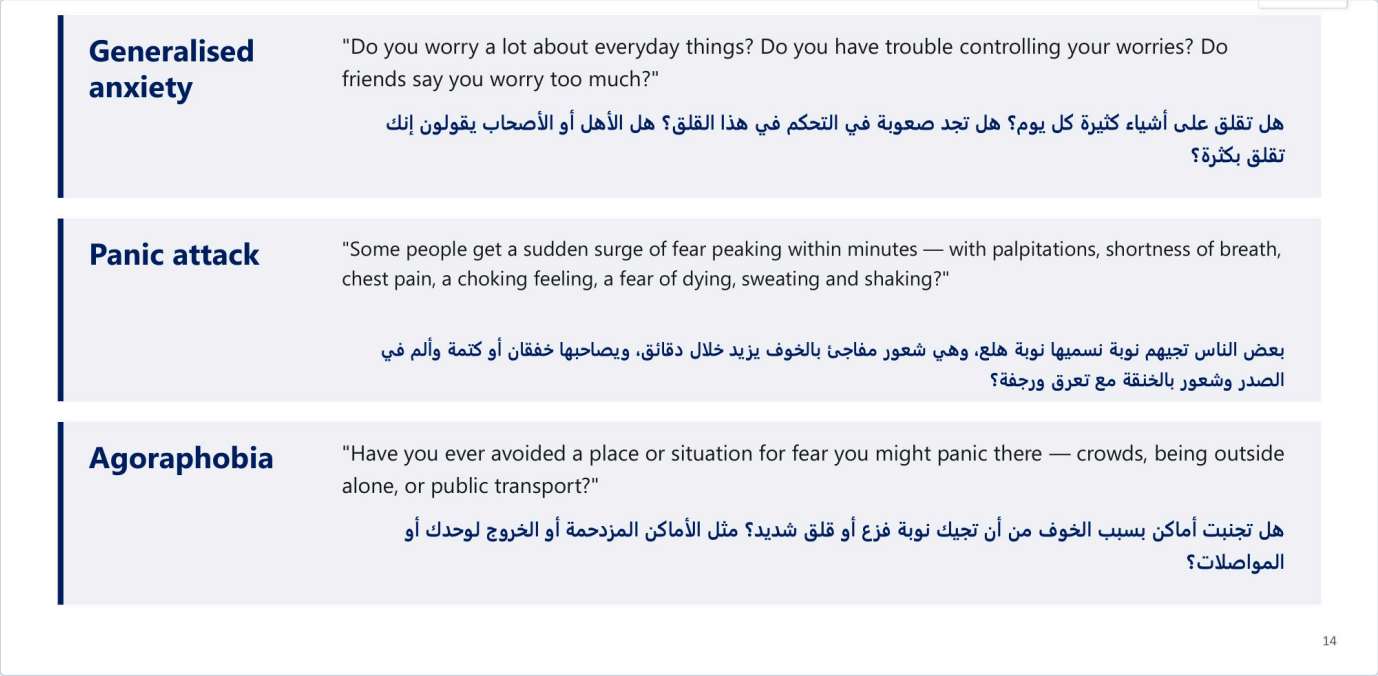
⭐ **Deliberately asking about ABSENT symptoms turns a history into a differential diagnosis.**

2.5 Screening Scripts (Bilingual) (Slides 14–16) ⭐

The slides give each script in **English and Gulf Arabic**. The Arabic is in the slide images below; the extracted Arabic text is unusable.

Disorder	English screening question
Generalised anxiety	"Do you **worry a lot about everyday things**? Do you have **trouble controlling** your worries? Do friends say you worry too much?"
Panic attack	"A **sudden surge of fear peaking within minutes**, with palpitations, shortness of breath, chest pain, choking, **fear of dying**, sweating, shaking?"
Agoraphobia	"Have you ever **avoided a place or situation for fear you might panic** there: crowds, being outside alone, public transport?"
Social anxiety	"Intense fear when you are the **centre of attention**: giving a speech, at a gathering, facing an authority figure?"
Specific phobia	"**Out-of-proportion** fear of (e.g. animals) that interferes with daily life?"
OCD	"Needing to **wash your hands** all the time because you feel dirty, constantly **checking**, or **annoying thoughts popping into your head** over and over?"
Psychosis	"Do things feel **not real**?" · "People **against you / after you**?" · "**Hear or see things** others don't?" · "Voices **outside or inside** your head?" · "Do they ever **command** you?"

- ⭐ **The feared CONSEQUENCE, not the situation, separates the anxiety disorders.** After a positive screen, always ask **WHY** the situation is avoided.
  - *[added]* For example, avoiding a mall: fear of panicking with no escape = **agoraphobia**; fear of being judged = **social anxiety**.
- ⭐ True **auditory hallucinations** are more often experienced as coming from **OUTSIDE** the head; patients may **look toward the voice**.
- ⭐ **ALWAYS ask about command hallucinations: it is a safety issue.**



Slide 14: GAD, panic, agoraphobia scripts (EN/AR)



Slide 15: Social anxiety and phobia scripts



Slide 16: OCD and psychosis scripts

2.6 Past Psychiatric, Medical & Drug History (Slide 17)

Past psychiatric	Medical, surgical & drug
Number of lifetime admissions and total episodes	Medical and surgical history
Ever had **ECT** or neurostimulation? Who is the treating provider?	⭐ **Psychiatric side-effects of drugs: steroids, interferon, some antihypertensives**
Diagnoses, treatments, **response to each**	—

Capture a long past history by **counting admissions and episodes** and noting treatment responses, not as a blow-by-blow narrative.

2.7 Substance History & CAGE (Slide 18) ⭐

For EACH substance: age of onset · amount and pattern · route · effects · withdrawal.

Substance	Specific asks
Tobacco	Packs/day; nicotine replacement (NRT)
Opioids	Type; IV/PO; **naloxone**
Alcohol	**Blackouts, binges, withdrawal, seizures**
Cannabis	**Paranoia?**
Others	Stimulants · benzodiazepines · ⭐ **CAFFEINE** (key if the patient is anxious)

CAGE	Question
Cut down	Ever felt you should cut down on your drinking?
Annoyed	Have people annoyed you by criticising your drinking?
Guilty	Ever felt guilty about your drinking?
Eye-opener	Ever had a drink first thing in the morning to steady your nerves?

⭐ **CAGE ≥ 2 positive = concern** → full assessment for alcohol use disorder.



Slide 18: Substance history and CAGE

2.8 Family History (Slide 19)

- **The family:** father, mother, siblings (health and relationships); home atmosphere; **monthly income; crowding**.
- **Family psychiatric history:** mental illness, substance use, personality disorder, ⭐ **COMPLETED SUICIDE**, ⭐ **response to specific treatments (guides drug choice)**.
- A strong family history **raises the pre-test probability** of mood and psychotic disorders.

2.9 Personal & Developmental History (Slide 20)

Early life	Growing up	Adult life
Prenatal, natal, postnatal; **milestones**; childhood "**neurotic traits**"	**Scholastic progress**; work record	Sexual and marital history; military/national service; **forensic history**

A **poor school and work record from early on** points to something **lifelong** rather than a new illness.

2.10 Premorbid Personality (Slide 21)

What was the person like BEFORE becoming unwell? This is the baseline against which the illness is measured.

8 domains:

1. sociability and relationships
2. impulse control
3. emotional stability
4. prevailing mood
5. character of thinking
6. religiosity
7. habits and hobbies
8. interests

⭐ **A marked CHANGE from premorbid personality is itself a red flag.** It also separates a **PD (lifelong)** from a **new illness (a change)**.

> 🔁 **Recap: The history.**
> - Reliable informant: lives with the patient, no conflict, no psychiatric disorder.
> - HPI must include **absent** symptoms.
> - Screen anxiety (ask WHY they avoid), OCD and psychosis (voices outside; **command** hallucinations).
> - Drug side-effects: steroids, interferon, antihypertensives.
> - Substances: each one in detail, plus caffeine. **CAGE ≥ 2**.
> - Family history: completed suicide, treatment response.
> - Premorbid personality = the baseline.
>
> ▶️ **Up next: Risk assessment.**

2.11 Risk Assessment (Slides 23–28) ⭐⭐

Strongest predictor (slides 23–24): ⭐ A previous suicide attempt. Other points from the answer slide:

- **marriage is protective**
- **completed suicide is higher in males**, despite **more attempts in females**

How to Ask (Slide 25)

- **Normalise, then ask directly:**
  - "Some people think of suicide when their mood is low; has this ever crossed your mind?"
  - "You're going through a painful loss; have thoughts of your own death ever crossed your mind?"
- ⭐ **Myth dispelled:** asking about suicide **does NOT plant the idea or increase risk**. It **RELIEVES distress** and opens the conversation; most patients are relieved to be asked.
- ⭐ **Screening for suicidal AND homicidal ideation is MANDATORY in every assessment.**

Current Safety & Past Attempts (Slide 26)

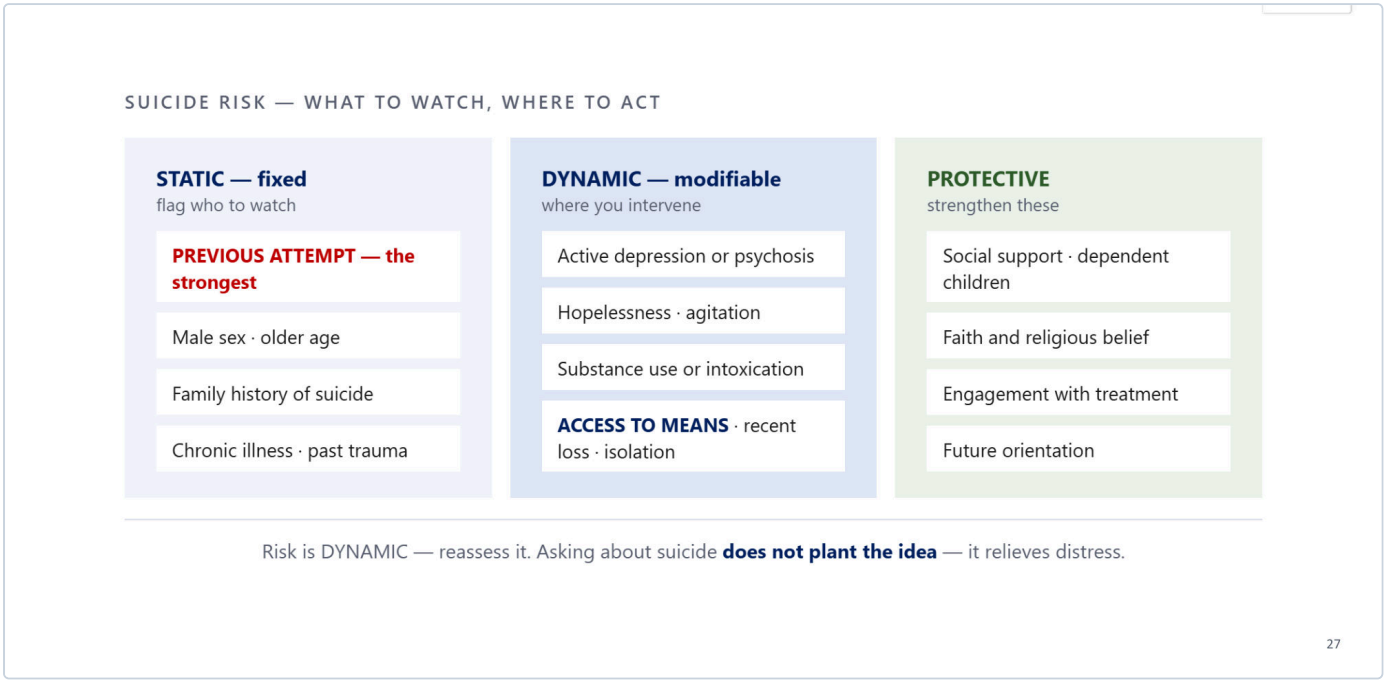
Current ("how serious")	Past ("the index attempt")
Is there a **PLAN**?	When, how, why?
Access to MEANS (firearms, medications, poisons)?	Was it with **expressed INTENT TO DIE**, vs an accidental overdose?
Intent to act immediately?	Any self-harm that could cause an "**inadvertent suicide**"?
Chronic, acute and imminent risk factors?	—

Risk & Protective Factors (Slide 27) ⭐

STATIC: fixed (flags who to watch)	DYNAMIC: modifiable (where you intervene)	PROTECTIVE (strengthen these)
⭐ **PREVIOUS ATTEMPT: the strongest**	Active **depression or psychosis**	**Social support**; **dependent children**
Male sex; **older age**	**Hopelessness**; **agitation**	**Faith and religious belief**
Family history of suicide	**Substance use or intoxication**	**Engagement with treatment**

STATIC: fixed (flags who to watch)	DYNAMIC: modifiable (where you intervene)	PROTECTIVE (strengthen these)
Chronic illness; **past trauma**	⭐ **ACCESS TO MEANS**; recent loss; isolation	**Future orientation**

Risk is DYNAMIC: reassess it.



Slide 27: Suicide risk, static vs dynamic vs protective

[added] Classic exam mnemonic **SAD PERSONS**:

- **S**ex (male)
- **A**ge (<19 or >45)
- **D**epression
- **P**revious attempt
- **E**thanol
- **R**ational thinking loss (psychosis)
- **S**ocial support lacking
- **O**rganised plan
- **N**o spouse
- **S**ickness

Homicide & Risk to Others (Slide 28)

- **Ask directly:**
  - threats driven by psychosis (**command hallucinations, persecutory delusions**)?
  - threats to **SPECIFIC, NAMED** individuals?
  - "**If you left now, would you want to hurt anyone?**"
- **Consider:**
  - access to **weapons**
  - **history of violence**
  - ⭐ **duty to warn or protect** an identifiable target, under the local legal framework
- Homicide risk is screened in **every** assessment, not only when prompted.

> 🔁 **Recap: Risk.**
>
> - Previous attempt = the strongest predictor. Males complete suicide more; females attempt more; marriage protects.
> - Ask directly after normalising; it doesn't plant the idea.
> - Assess plan, means, intent, and the index attempt's intent.
> - Static vs dynamic vs protective factors.
> - Homicide: named targets, weapons, duty to warn.
>
> ▶️ **Up next: Organic red flags and medical mimics.**

2.12 Organic Red Flags (Slides 30–32) ⭐⭐

Case (slides 30–31): a 58-year-old with no psychiatric history develops, over **2 weeks**:

- **visual hallucinations**
- **fluctuating confusion, worse at night**
- disorientation
- **mild fever**

→ ⭐ **Investigate for an organic cause (delirium)**, not an antipsychotic or "late-onset schizophrenia".

The presentation	The course
⭐ **New onset over ~40** with **NO** prior psychiatric history	**Rapid, atypical, or treatment-resistant** course
Fluctuating consciousness or disorientation (delirium)	**Cognitive decline** out of keeping with the mood
⭐ **VISUAL, olfactory or tactile** hallucinations	Always take a full medical and drug history and **EXAMINE**
Focal neurological signs; seizures; **abnormal vital signs**	—

⭐ **Golden rule: exclude an organic cause BEFORE diagnosing a primary psychiatric disorder**, especially with late onset or atypical features.

The presentation

- New onset over about 40 with NO prior psychiatric history
- Fluctuating consciousness or disorientation — delirium
- VISUAL, olfactory or tactile hallucinations
- Focal neurological signs; seizures; abnormal vital signs

The course

- Rapid, atypical, or treatment-resistant course
- Cognitive decline out of keeping with the mood
- Always take a full medical and drug history, and EXAMINE the patient

The golden rule: exclude an organic cause BEFORE diagnosing a primary psychiatric disorder — especially with late onset or atypical features.

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Slide 32: Organic red flags

2.13 Medical Mimics (Slide 33)

Endocrine / metabolic	Neurological	Infective / inflammatory	Drugs / other
Thyroid disease; **Cushing's, Addison's**	**Epilepsy, esp. temporal lobe**	**Autoimmune (anti-NMDA)** and **Hashimoto's encephalitis**	**Medication and substance** effects
Hyper/hypocalcaemia, PTH	**MS, Huntington's, TBI**	**Neurosyphilis, HIV, Lyme, cryptococcal meningitis**	**Delirium; catatonia**
Low Na / K / Mg; **hepatic encephalopathy**; **porphyria**	**Kleine–Levin, Klüver–Bucy**	**SLE**; **paraneoplastic** syndromes	**Niemann–Pick C; Moyamoya; post-MI**

You need not memorise every entry, but **reach for this list whenever red flags are present**.

Endocrine / metabolic

- Thyroid disease; Cushing's, Addison's
- Hyper- or hypocalcaemia, PTH
- Low Na / K / Mg; hepatic encephalopathy; porphyria

Neurological

- Epilepsy, especially temporal lobe
- MS, Huntington's, traumatic brain injury
- Kleine–Levin, Klüver–Bucy

Infective / inflammatory

- Autoimmune (anti-NMDA) and Hashimoto's encephalitis
- Neurosyphilis, HIV, Lyme, cryptococcal meningitis
- SLE; paraneoplastic syndromes

Drugs / other

- Medication and substance effects
- Delirium; catatonia
- Niemann–Pick C; Moyamoya; post-MI

You are not expected to memorise every entry — you are expected to reach for this list whenever the red flags on the previous slide are present.

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Slide 33: Medical mimics

2.14 The Difficult Interview (Slide 34) (OSCE)

Situation	Approach
Guarded / suspicious	Go slowly, be transparent, ⭐ **do not challenge delusions**; build safety first
Hostile / agitated	Calm, non-confrontational; ⭐ **ensure your own safety and an exit**; set limits
Mute / withdrawn	Patience, open posture, **collateral history**; ⭐ **consider catatonia or depression**
Over-talkative	Gently structure and redirect with **closed questions**
Seductive / boundary-testing	Maintain professional boundaries; **use a chaperone**; **document**

Common thread: **manage the interaction**, don't just push through your question list.



Slide 34: The difficult interview

2.15 Physical Examination and Closing (Slides 36–37)

- **Examination:**
  - general, including **vital signs**
  - **neurological** (motor, sensory, reflexes, ⭐ **FUNDUS**)
  - cardiac and chest
- **Why it matters:**
  - screens for organic causes
  - provides a ⭐ **BASELINE before medication: weight, BP, ECG**
- **Closing:**
  - "Did we cover the main concerns you hoped to discuss today?"
  - "Any thoughts on how we might help?"
  - **summarise and agree the plan**
  - **thank** the patient for sharing a "snippet" of their life
  - a collaborative close reinforces **rapport and adherence**

2.16 Integrative Case: Mr. S, 62 (Slides 38–39) ⭐

Presentation:

- retired teacher, brought by his **son**, says nothing is wrong
- over **3 weeks**: suspicious that neighbours enter the house; twice **saw children in the living room** who weren't there
- **more confused in the evenings**, poor sleep, some mornings almost himself
- no psychiatric history
- **prednisolone 6 weeks ago**; **tramadol** for back pain
- drinking less, weight loss
- **mildly febrile**, unsure of the date, asks **twice** why he is there

Question	Model answer (lecture)
1. Whose account?	**The son.** Collateral is essential when insight is lost. He lives with the patient, has no conflict of interest and no psychiatric disorder
2. Organic red flags?	**New onset at 62**, no history · **visual hallucinations** · **fluctuating consciousness, worse at night** · disorientation · **fever**
3. Working diagnosis?	⭐ **DELIRIUM: organic until proven otherwise.** Don't diagnose late-onset psychosis. Exclude **infection, drugs (prednisolone, tramadol), dehydration, electrolyte disturbance**
4. Priorities now?	Full medical and drug history, substance history, then **examination** (vitals, neurological including **fundus**, chest) + investigations
5. Risk before he leaves?	**Suicide AND homicide.** His persecutory beliefs about the neighbours make **risk to others** a live question; ask directly

Twist: if the same picture had developed over **3 years** rather than 3 weeks → **dementia**, with a different work-up.

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3. High-Yield Comparison Tables

Table A: Primary Psychiatric vs Organic Presentation

Feature	Primary psychiatric (e.g. schizophrenia)	Organic (delirium, etc.)
Age of onset	Usually young (teens–30s)	**> 40, no prior history**
Hallucinations	**Auditory** (voices, from outside)	**Visual, olfactory, tactile**
Consciousness	Clear	**Fluctuating**, disoriented, worse at night
Onset/course	Insidious	**Rapid**, atypical, treatment-resistant
Vitals/neuro	Normal	Fever, abnormal vitals, focal signs, seizures
Cognition	Matches the illness	Decline out of keeping with mood

Table B: Delirium vs Dementia (from the case twist) *[mostly added]*

	Delirium	Dementia
Onset	**Days–weeks** (lecture: 3 weeks)	**Months–years** (lecture: 3 years)
Consciousness	**Fluctuating**	Clear until late
Attention	Markedly impaired	Relatively preserved early
Hallucinations	Visual, common	Less common early (except Lewy body)

	Delirium	Dementia
Reversibility	Usually reversible; find the cause	Usually progressive

Table C: Suicide Risk Assessment Summary

Layer	Content
Ask	Normalise → direct question; **always**, including homicide
Current	Plan · means · immediate intent
Past	Index attempt: when, how, why, **intent to die**
Static	**Previous attempt**, male, older, FHx suicide, chronic illness, trauma
Dynamic	Depression/psychosis, hopelessness, agitation, substances, **means**, loss, isolation
Protective	Support, dependent children, **faith**, treatment engagement, future orientation

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4. Rules, Exceptions, Patterns

- **History + MSE = the investigation.** There is no diagnostic test for most disorders.
- **Bereavement and culturally approved responses are not disorders**, nor is social deviance **unless** it stems from individual dysfunction.
- **Ask about absent symptoms** to build the differential.
- **Insight lost** (mania, psychosis, dementia) → collateral is mandatory.
- **Feared consequence**, not the situation, distinguishes the anxiety disorders.
- **Voices from outside** + patient looking toward them = typical true hallucination. **Command hallucinations = safety issue.**
- **Previous attempt** is the strongest suicide predictor. **Males complete, females attempt**; marriage protects.
- **Asking about suicide is safe** and mandatory; so is asking about homicide.
- **Late onset (>40), visual/olfactory/tactile hallucinations, fluctuation, fever, focal signs** → organic first.
- **Steroids, interferon, some antihypertensives** cause psychiatric side-effects.
- **Examination gives a baseline** (weight, BP, ECG) before psychotropics.
- **Change from premorbid personality** = new illness; lifelong pattern = personality/neurodevelopmental.

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5. Clinical Correlations

- **Elderly patient with new "psychosis" on steroids or opioids** → delirium. Check infection, drugs, hydration, electrolytes.
- **Anxious patient** → ask about **caffeine** (and thyroid; *[added]* phaeochromocytoma and hypoglycaemia are rarer mimics).
- **Psychotic patient with persecutory delusions about named neighbours** → homicide risk; duty to protect.
- **Mute patient** → consider **catatonia** (and depression); get collateral.
- **Patient with a strong family history** of good response to a specific antidepressant → it guides drug choice.
- **Endocrine overlap:** hypercalcaemia (stones, bones, groans, **psychic moans**), Cushing's (depression, psychosis), Addison's (apathy, depression), thyrotoxicosis (anxiety).

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6. Rapid Revision (Last-Day)

1. **Psychiatry:** Reil, 1808; psykhē + iatreia.
2. **Mental disorder** = syndrome + dysfunction + distress/disability. **Not** bereavement or social deviance (unless dysfunction).
3. **DSM-5-TR (2022) and ICD-11 (2022)**. Diagnosis is syndrome-based; the **history and MSE are the investigation**.
4. **Setting:** private, **L-shaped seating**, open → focused questions, minimal notes.
5. **9 sections:**
    1. personal data
    2. presenting complaint
    3. HPI
    4. past psychiatric history
    5. past medical/drug history
    6. substances
    7. family history
    8. personal/developmental history
    9. **premorbid personality**

Then MSE → exam → investigations → formulation. 6. **Reliable informant:** lives with the patient, no conflict of interest, no psychiatric disorder. 7. **HPI:** symptoms present **and absent**. 8. **Screen:** GAD, panic, agoraphobia, social anxiety, phobia, OCD, psychosis. Ask **why** they avoid; ask about **command** voices. 9. **CAGE ≥ 2** = alcohol concern. Ask about **caffeine** in anxiety. 10. **Family history: completed suicide** + **treatment response**. 11. **Strongest suicide predictor = previous attempt.** 12. Asking about suicide **doesn't plant the idea**. Suicide + homicide screening is **mandatory**. 13. **Current risk:** plan, means, intent. **Index attempt:** intent to die? 14. **Static / dynamic / protective** factors (faith, children, support, engagement, future orientation). 15. **Organic red flags:** onset >40, fluctuating consciousness, non-auditory hallucinations, focal signs, abnormal vitals, atypical course. 16. **Difficult interview:** don't challenge delusions; keep an exit; consider catatonia; use closed questions; use a chaperone. 17. **Examination** (including fundus) = organic screen + **baseline** (weight, BP, ECG).

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7. Exam-Style Questions

Q1 (lecture, slides 23–24). Which most strongly predicts suicide? A. Stress at work · **B. Previous suicide attempt** · C. Being married · D. Female sex

> **Answer: B** (lecture key).
> - Previous attempt is the single strongest predictor.
> - Marriage is **protective**.
> - Males **complete** suicide more often, though females attempt more.

Q2 (lecture, slides 30–31). A 58-year-old with no psychiatric history develops, over 2 weeks, visual hallucinations, fluctuating confusion worse at night, disorientation and mild fever. First priority? A. Start an antipsychotic · **B. Investigate for an organic/medical cause** · C. Psychotherapy · D. Diagnose late-onset schizophrenia

Answer: B (lecture key).

- Late onset, no history, **visual** hallucinations, **fluctuating** consciousness and fever = **delirium**.
- Find and treat the cause.

Q3. A medical student argues that asking a depressed patient about suicide may "put the idea in her head." Best response? A. Agree; ask only if she raises it · B. Ask only if there is a previous attempt · **C. Asking directly does not increase risk and relieves distress; it must always be asked** · D. Ask the family instead

Answer: C (slide 25).

- It's a myth that asking plants the idea.
- Screening for suicidal **and homicidal** ideation is mandatory in every assessment.

Q4. A man brings his wife, who has new grandiose ideas and has not slept for a week. She insists she is fine. Which informant feature does **NOT** make him reliable? A. Lives with her · B. Has no conflict of interest · C. Has no psychiatric disorder · **D. He is the person who brought her to clinic**

Answer: D *[reasoning added; framed from slide 12]*.

- The three reliability criteria: **close to and living with** the patient, **no conflict of interest**, **no psychiatric disorder**.
- Simply bringing her in is not a criterion.
- Collateral is essential here because insight is limited (mania).

Q5. A patient's CAGE answers: he has thought he should cut down and he drinks in the morning to steady his nerves. Interpretation? A. Normal · B. Diagnostic of alcohol dependence · **C. ≥ 2 positives: a concern needing full assessment** · D. Only significant if ≥ 3

Answer: C (slide 18).

- CAGE is a **screen**, not a diagnosis.
- ≥ 2 positives warrants full assessment for alcohol use disorder.

Lecture board one-liners (slide 40):

#	Question	Answer
1	Strongest predictor of completed suicide?	**Previous suicide attempt**
2	Does asking about suicide increase risk?	**No**; always ask
3	CAGE screens for?	**Alcohol use disorder (≥ 2 = concern)**
4	New visual hallucinations + fluctuating consciousness at 60?	**Delirium / organic cause**; investigate first
5	Reliable informant?	Close to and living with the patient, no conflict of interest, no psychiatric disorder

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8. Exam Pitfalls & Traps

Trap	Correct thinking
"Asking about suicide may trigger it"	❌ Myth; it relieves distress. Mandatory every time
Strongest suicide predictor = depression / hopelessness	❌ **Previous attempt**
Females complete suicide more (they attempt more)	❌ **Males complete** more; females attempt more
Skipping homicide risk when the patient "seems calm"	❌ Screen in **every** assessment; named targets → duty to protect
New hallucinations at 58 → antipsychotic / late schizophrenia	❌ **Delirium until proven otherwise**; investigate
Visual hallucinations → schizophrenia	❌ Visual/olfactory/tactile → **organic** red flag
Face-to-face seating	❌ **L-shaped**; face-to-face feels like an interrogation
Grief = depressive disorder	❌ An expectable response to loss is **not** a disorder by definition
Only asking about symptoms present	❌ Ask about **absent** symptoms to exclude the differentials
Relying on the patient alone in mania/psychosis/dementia	❌ **Collateral** from a reliable informant is essential
Challenging a guarded patient's delusions	❌ Don't challenge; go slowly, build safety
Skipping the physical exam in psychiatry	❌ Organic screen + **baseline** (weight, BP, ECG) before drugs
CAGE positive = diagnosis	❌ Screen only; ≥ 2 = **concern** → full assessment
ICD-10 is current	❌ **ICD-11** in effect since 2022